

City and Hackney Systemic Lupus Erythematosus guidelines

Systemic Lupus Erythematosus (SLE) is a rare chronic multisystem inflammatory autoimmune disease. Incidence 9/100,000 and more common in Afro-Caribbeans.

Clinical Features

The primary symptoms and signs are:

- Fatigue (Often extreme i.e having to go to bed at 19:00 or 20:00)
- Arthralgia (Typically pain in multiple small joints but swelling often absent)
- Rashes (Not dermatitis and photosensitivity is more specific for SLE)

Other features may include:

- Raynaud's phenomenon (Very Common)
- Lymphadenopathy (Common with flares)
- Recurrent mouth ulcers (Common)
- Alopecia (Common with flares)
- Weight loss / Fever (Only very active disease)
- Renal: nephritis, nephrotic syndrome, renal failure (Common with severe disease)
- Neurological: seizures, peripheral or central neuropathy, memory loss, headaches and migraine, Stroke
- Psychiatric: psychosis and depression
- Haematological: haemolytic anaemias, neutropaenia, lymphopenia, thrombocytopenia (Uncommon)
- Pulmonary: pleurisy (Very common with flares), fibrosing alveolitis (Uncommon)
- Cardiovascular: pericarditis (Common), increased risk of ischaemic heart disease
- Arterial and venous thromboembolic disease if antiphospholipid syndrome also present

Triggers

- Exposure to sunlight
- Hormonal changes e.g. puberty or pregnancy
- Medications i.e. Lymecycline, Oestrogen containing contraceptives

Investigations

If SLE is suspected from clinical features, the following should be requested:

Primary Care investigations

ANA

- ANA (95% sensitivity, specificity 57%) - SLE with a negative ANA is unlikely. A low titre ANA (1:40 - 1:80) is present is approximately 10% of the normal population.
- If ANA is negative or low titre and there is a high clinical index of suspicion please discuss with rheumatology via the rheumatology advice and guidance service (e-RS)
- If ANA titre 1:160 or more: refer to rheumatology
- Urinalysis: To check for nephropathy (haematuria and proteinuria) NB if large amounts of protein and blood are present and renal function is deranged, parallel referral to renal medicine at the Royal London is recommended
- FBC: Anaemia / Cytopenia
- U&Es: Renal impairment
- LFTs: Liver typically spared in SLE
- ESR and CRP (Classically in SLE a normal or minimally elevated CRP i.e 5-15 with high ESR)

In addition to a normal rheumatology referral rheumatology can be contacted via either:

- Rheumatology Advice & Guidance via e-RS
- Call Rheumatology Secretaries 0208 510 7612 and ask for a call back from a consultant
- If patient is pregnant, a referral made in parallel to obstetric medicine should be made

Rheumatology will investigate further and confirm the diagnosis with the following investigations:

Investigation in secondary care

- ENA panel (Ro, La, Jo-1, Sm, RNP, Scl-70)
- Anti-dsDNA antibodies
- Complement C3 and C4
- Anti-phospholipid antibodies

Management:

If confirmed, patients will be kept under lifelong rheumatology review and management.

Management options in secondary care:

- Hydroxychloroquine (Shared Care Guideline)
- Corticosteroids
- Azathioprine (Shared Care Guideline)
- Mycophenolate Mofetil (Shared Care Guideline)
- Cyclophosphamide (Hospital Only)
- Rituximab (Hospital Only)

Prescribing of some of these drugs may be shared with GPs under shared care guidelines,
<http://www.cityandhackneyccg.nhs.uk/gp/GPResources/Prescribing/shared-care.htm>

Management in primary care:

Further areas where GPs can give important input are as follows:

- Ensure patients have relevant patient information regarding the condition and its treatment:
<https://www.arthritisresearchuk.org/arthritis-information/conditions/lupus.aspx>
- Advise sun protection
- Vitamin D supplementation (as avoiding sunlight)
- Screening and treatment of depression
- Drugs such as corticosteroids and immunosuppressants also increase the risk of opportunistic and other serious infection. Immunosuppressant medication may need to be stopped: please discuss with Rheumatology if in doubt.
- Vaccination – Pneumococcal and Flu in patients on immunosuppressants
- Discuss methods of contraception if required (see Shared Care Guidelines)
- Control of cardiovascular risk factors (as risk of IHD is up to 50 times higher than average):
 - Smoking cessation
 - Healthy balanced diet
 - Healthy weight
 - Regular exercise – at least 150 minutes a week
 - Limit alcohol intake to within recommended guidelines (14 units a week)
 - Yearly HbA1C, lipids, BP check and QRISK calculation

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